

PALM MUTUAL OF FLORIDA

MEDICAL COVERAGE GUIDELINE

Policy PM-02-41000-18

Endovascular Stenting for Lower Extremity Peripheral Artery Disease

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Original effective date	03/01/2023
Reviewed	09/30/2025
Revised	10/15/2025
Lines of business	Commercial and Medicare Advantage, as a synthetic test policy only

Description

Atherosclerotic narrowing of the iliac, femoral, and popliteal arteries causes claudication and, when severe, rest pain, ischemic ulcer, or gangrene. Walking programs, an antiplatelet drug, a statin, and smoking cessation are the starting point for claudication. Stenting is reserved for critical limb ischemia or for claudication that remains lifestyle-limiting after a real trial of that care. This policy addresses iliac stent placement and femoropopliteal stent placement. It does not address open bypass, tibial intervention, or intravascular lithotripsy.

Position statement

Femoropopliteal stent, CPT 37226

Unilateral femoropopliteal stent placement (CPT 37226), including angioplasty in the same vessel when it is part of the stent procedure, meets medical necessity when **all** of the following are documented:

Duplex, CTA, MRA, or catheter angiography within 6 months shows a de novo or restenotic atherosclerotic stenosis of at least 70 percent in the superficial femoral artery or popliteal artery that is the target of the stent. The report must name the artery and the laterality. A lesion under 50 percent is not a target; **AND**

An ipsilateral ankle-brachial index of 0.90 or lower, or a toe-brachial index of 0.70 or lower, measured within 6 months; **AND**

At least **one** of the following indications:

1. Critical limb ischemia in that leg: ischemic rest pain, a nonhealing ischemic ulcer, or gangrene, attributed to the target lesion. A 12-week exercise trial is **not** required when this indication is met; **OR**
2. Lifestyle-limiting claudication, meaning calf, thigh, or buttock pain that restricts employment or limits walking to less than two blocks, **AND** a documented trial of conservative therapy lasting at least 12 weeks. That trial must include a structured walking program and at least one antiplatelet or cilostazol, without adequate relief. If the patient currently smokes, smoking-cessation counseling during the trial must be documented.

Iliac stent, CPT 37221

Unilateral iliac stent placement (CPT 37221) uses the same ankle-brachial or toe-brachial requirement and the same choice of indications. The imaging requirement is a stenosis of at least 70 percent in the common or external iliac artery, named and lateralized, within 6 months. A patent iliac artery is not a reason to select this pathway.

Exclusions and investigational procedures

Exclusion. Stenting to treat asymptomatic peripheral artery disease, or to treat a lesion measured at under 50 percent, does not meet medical necessity.

Investigational. Intravascular lithotripsy, including HCPCS C9764 and C9765, does not meet medical necessity under this policy. Calcification on the imaging report does not convert lithotripsy to a covered stent code.

Open surgical bypass, atherectomy-only procedures, and tibial or pedal revascularization are outside this policy. A code that does not appear in the coding table is not covered by this policy.

Billing and coding

Code	Description	Coverage
37221	Revascularization, iliac artery, with stent, unilateral, including angioplasty in the same vessel when performed	Covered if criteria met
37226	Revascularization, femoral or popliteal artery, with stent, unilateral, including angioplasty in the same vessel when performed	Covered if criteria met
C9764	Intravascular lithotripsy, iliac artery, with or without stent	Investigational
C9765	Intravascular lithotripsy, femoral or popliteal artery, with or without angioplasty	Investigational

Diagnosis codes that support medical necessity when the criteria above are also met include I70.211 and I70.212 (claudication), I70.221 and I70.222 (rest pain), I70.231 through I70.235 and I70.241 through I70.245 (ulceration), and I70.261 and I70.262 (gangrene).

Documentation to submit

History and physical with laterality, walking limitation or rest-pain and tissue-loss description, and smoking status.

The ABI or TBI report with the number, the leg, and the date.

The imaging report with percent stenosis, vessel name, and date. The study must be within 6 months of the request.

For claudication requests: the walking-program start date, the follow-up that shows at least 12 weeks, the antiplatelet or cilostazol, and cessation counseling if the patient smokes. Do not substitute a plan to start exercise later.

The procedure order with CPT or HCPCS, artery, and laterality.

Definitions

Critical limb ischemia means ischemic rest pain, a nonhealing ischemic ulcer, or gangrene. Claudication alone is not critical limb ischemia.

Structured walking program means a prescribed program with a start date and at least one follow-up documenting adherence. Advice to walk more, without dates, does not establish the 12-week trial.

Lifestyle-limiting means the symptom restricts work or limits continuous walking to less than two blocks. Mild discomfort that does not change activity is not enough.

References

Published papers are named so this guideline has a bibliography. This policy is not those documents.

Conte MS, et al. Society for Vascular Surgery practice guidelines for atherosclerotic occlusive disease of the lower extremities. J Vasc Surg. 2015;61:2S-41S.

Gerhard-Herman MD, et al. 2016 AHA/ACC guideline on the management of patients with lower extremity peripheral artery disease. Circulation. 2017;135:e726-e779.

Aboyans V, et al. 2017 ESC guidelines on the diagnosis and treatment of peripheral arterial diseases. Eur Heart J. 2018;39:763-816.